Bacterial Skin Infection

By:

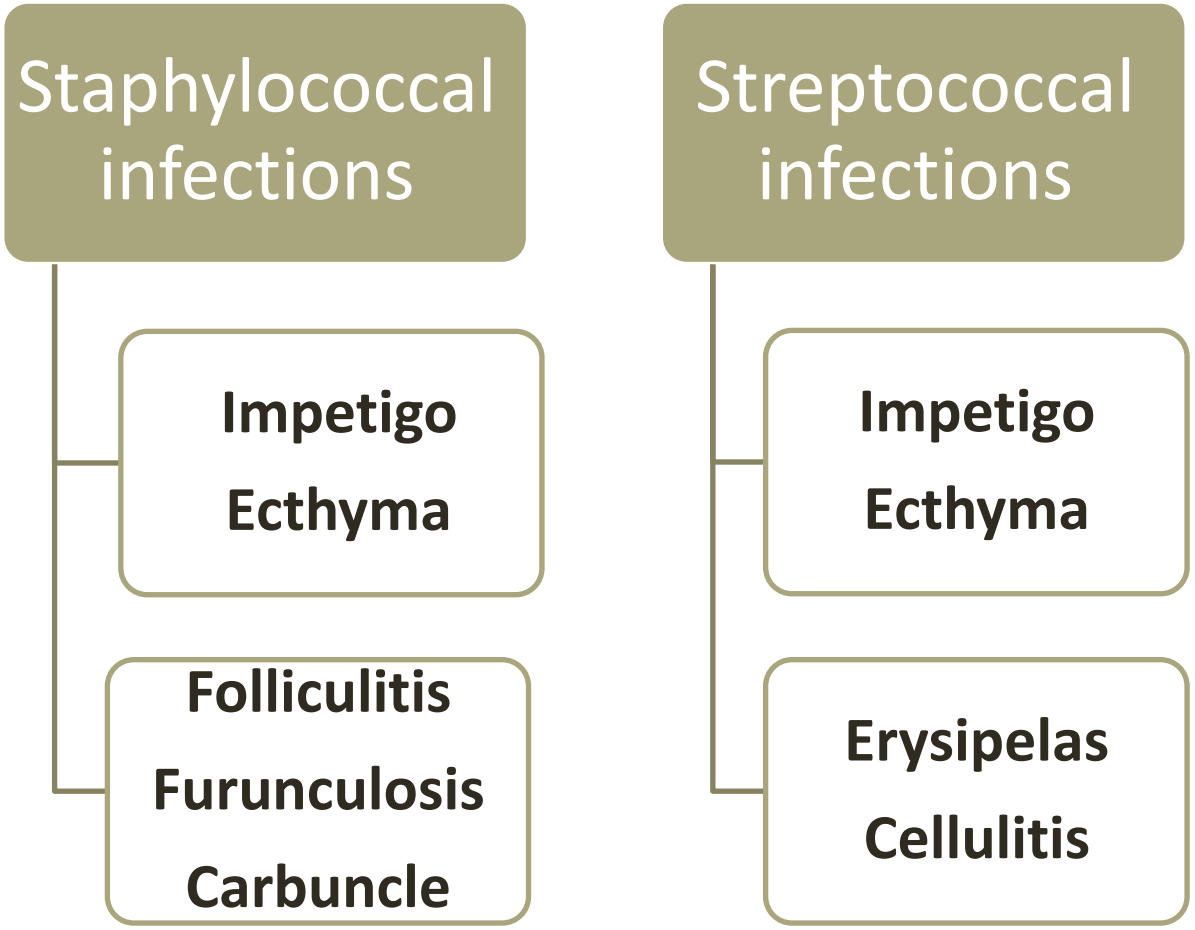
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Bacterial skin infections are very common diseases

- **Causes:**



Impetigo

- Infectious superficial pyogenic infection of skin.
- AE: Staphylococci & streptococci
- More common in childern.
- Occurs mostly in summer.

Clinical types:

- 1- Non-bullous impetigo
- 2- Bullous impetigo

Non bullous impetigo



Bullous impetigo



Non bullous impetigo





Clinical picture of non bullous type

Thin walled vesicle on erythematous base

Rupture & exude serum

then dry to form yellowish crust

The crust separate leave erythema

Fade without scar

Site: common on face, neck, scalp, hand & extremities





















Bullous impetigo

bullae are fewer in number and much larger.

occur anywhere on the body.

Common in neonates.



Acute glomerulonephritis may occur as **a complication of group A beta-hemolytic strept.**

Treatment:

1- Control predisposing factors (itching, scabies, pediculosis)

2- **Topical antibiotics (alone in localized infections).**

3- Systemic antibiotics (in severe infection and with lymphadenopathy).

Cellulitis & Erysipelas

- **Deep bacterial** infection of the skin.

- **Clinical picture**:
- Fever, chills & malaise occur. The lesion show the 4 cardinal signs of inflammation (**hotness, redness, pain & swelling**) & lymphangitis and lymphadenopathy.

- **Site:** **face** & **legs** are commonly affected.

Cellulitis





Cellulitis





Erysipelas





Cellulitis & Erysipelas

- **Predisposing factors**: wounds, fissures, ulcer or inflammatory lesion.

- **Complication:** rare
- Acute glomerulonephritis, lymphadenitis & subacute bacterial endocarditis.

- **Treatment:** Systemic penicillins & cephalosporins.


A+

Case 1

- A female child 6 years old is complaining of multiple painful **yellowish crusts on erythematous skin of her chin & cheeks of** 1 wk duration. She has **fever & submental lymphadenopathy** in addition.
- 1- What's your provisional **diagnosis?**
- 2-What's the suitable **treatment** ?
- 3-What's the possible **complication to this case?**

Case 2

- A man 50 years old, diabetic, he is complaining of **hot, red painful swollen erythema** of his Rt. Leg. of 5 days duration. He had history of an accidental trauma leaved a wound in the same leg 10 days ago. He had **fever, headache & malaise.**
- 1- What's your diagnosis?
- 2-How to treat this case?


Thank You